

SKIN & LASER

Vascular & Pigmented Lesion Treatment

Target selection, endpoints, and the lesions that must never be treated without diagnosis

DOCUMENT SKN-006	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO All laser operators	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

This protocol covers targeted treatment of vascular and pigmented lesions. Its most important section is the first one: which lesions must be seen by a dermatologist before any device is used on them.

THE SINGLE GREATEST RISK IN THIS PROTOCOL

Treating a melanoma as though it were a lentigo destroys the lesion's diagnostic features, delays diagnosis, and can be catastrophic for the patient. Aesthetic staff are not positioned to distinguish benign pigmented lesions from malignant ones, and confidence developed over years of treating sun spots is not a substitute for a diagnosis.

2 Mandatory Pre-Treatment Assessment

Every pigmented lesion considered for treatment is individually assessed and documented. Blanket treatment of 'sun spots' across a field without lesion-by-lesion assessment is not acceptable practice.

Never treat — refer for dermatological assessment first

- Any lesion with asymmetry, irregular or notched borders, colour variation, diameter growth, or evolution in size, shape, colour or symptom.
- Any lesion the patient reports as new, changing, itching, bleeding or crusting.
- Any lesion that looks different from the patient's other lesions.
- Any raised, nodular or ulcerated pigmented lesion.
- Congenital naevi and atypical or dysplastic naevi.
- Any lesion you are uncertain about — uncertainty is itself the referral criterion.
- Any lesion in a patient with a personal or family history of melanoma, without prior dermatological clearance.
- Pigmented lesions on acral sites, nail beds or mucosa.

DOCUMENTED CLEARANCE

Where a lesion has been assessed by a dermatologist and cleared for cosmetic treatment, obtain that in writing and file it. A patient reporting that their doctor said it was fine is not clearance. Re-refer if the lesion changes subsequently.

3 Target & Device Selection

FRAMEWORK — CONFIRM AGAINST YOUR DEVICE IFU

Target	Typical approach	Endpoint
Superficial pigmented lesions — lentigines, ephelides	Shorter wavelengths strongly absorbed by melanin; Q-switched or picosecond pulses.	Immediate ash-white or grey change, or subtle darkening depending on device.
Deeper dermal pigment	Longer wavelengths with short pulse durations.	Subtle whitening. Under-treat rather than over-treat; multiple sessions expected.
Fine facial telangiectasia	532 nm or pulsed dye.	Vessel darkening, transient disappearance, or mild purpura depending on settings.
Diffuse erythema and rosacea	Pulsed dye or IPL where skin type permits.	Mild transient purpura or erythema per the chosen settings.
Larger or deeper vessels, leg veins	1064 nm Nd:YAG.	Vessel contraction or disappearance. Higher risk of thermal injury; conservative settings essential.
Cherry angiomas	Vascular-targeting wavelength, single pulses.	Immediate greying or contraction of the lesion.

TEST SPOT EVERY TIME

Lesion work is treated as a new treatment for test-spotting purposes whenever the target type, device or facial area changes. Treat one representative lesion, review at the interval set by your medical director, then proceed.

4 Procedure

- 1 Confirm assessment and any dermatological clearance** is documented before powering the device.
- 2 Photograph the lesions** individually at standardized angles and lighting. This is both a clinical record and your evidence of what was present beforehand.
- 3 Map and count** the lesions to be treated. Agree the treatment field with the patient explicitly.
- 4 Eyewear on everyone** per SKN-008. Metal eye shields for lesions near the orbital rim.
- 5 Confirm parameters** against the IFU for the target and skin type.
- 6 Treat lesion by lesion** with single pulses. Do not stack pulses on one lesion unless the IFU directs it and the endpoint has not been reached.
- 7 Observe the endpoint** after each pulse. Stop on blistering, greying of surrounding normal skin, or pinpoint bleeding.
- 8 Cool** and apply a bland emollient.
- 9 Written aftercare** including the expected darkening and crusting sequence.

10 **Record** lesion locations, count, device, parameters and endpoints.

SET EXPECTATIONS ABOUT CRUSTING

Treated pigmented lesions typically darken, form a fine crust and shed over one to two weeks. Patients who were not warned believe the treatment made things worse and often pick, which causes scarring and PIH. Warn them in writing and show them a photograph of the expected appearance.

5 **Complications & Aftercare**

Complication	Response
Post-inflammatory hyperpigmentation	Follow the pathway in SKN-009. More likely in higher Fitzpatrick types and with sun exposure during healing.
Hypopigmentation	May be delayed and may be persistent. Photograph, notify the medical director, no further treatment to the area pending review.
Blistering or crusting beyond expectation	Manage per SKN-009. Photograph and escalate.
Purpura	Expected with some vascular settings. Warn in advance; resolves over one to two weeks.
Incomplete clearance	Common and expected. Plan a series; do not increase energy to force a result in one session.
Lesion recurrence	Pigmented lesions recur with sun exposure. Emphasize photo-protection and set maintenance expectations at the outset.
A treated lesion behaving unexpectedly	Stop treating it. Refer for dermatological assessment immediately.

- No picking or scrubbing crusts — they must shed on their own.
- Strict sun protection during healing and ongoing.
- No heat, sauna or vigorous exercise for the specified period.
- Bland skincare until fully healed.
- Review appointment before further sessions in a series.

6 Quick Reference

QUICK REFERENCE CARD

Before treating any lesion

1. **Assess every lesion individually.** No blanket field treatment.

2. **Asymmetry, irregular border, colour variation, change, new, itching, bleeding → refer.**

3. **Uncertain? That is the referral criterion.** Do not treat.

4. **Dermatological clearance in writing,** filed. Not verbal report.

5. **Photograph before,** standardized, every lesion.

6. **Test spot** for any new target, device or area.

7. **Single pulses. Watch the endpoint.** Do not stack to force clearance.

8. **Warn about darkening and crusting** or they will think it went wrong.

Print, laminate and post at the point of care

7 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

PRACTICE NAME

LOCATION / SITE

MEDICAL DIRECTOR — PRINTED NAME & CREDENTIALS

LICENSE NUMBER & STATE

SIGNATURE

DATE ADOPTED / NEXT REVIEW DATE

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SKN-006 Vascular & Pigmented Lesion Treatment — MedSpa Standards v1.0 · January 2026

Page 4 of 4